

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

Meningococcal ACWY Conjugate Vaccine (Nimenrix, MenQuadfi or Menveo)

Standalone PGD, separated from the combined travel document

This PGD was previously part of a single combined document covering Japanese encephalitis, rabies and meningococcal ACWY vaccination. That was raised by Pharmacy Plus Health on 21 August 2026: a pharmacy adopting one of the three was signing for all three, including vaccines it may not stock and in which its staff may not be trained. The three have been separated so that each can be adopted, trained for and signed off on its own merits.

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products supplied, as well as BNF advice and any applicable national guidance • All users must be competent in intramuscular vaccination technique and in the recognition and management of anaphylaxis • All users must be able to distinguish between the three licensed MenACWY products, whose licensed age ranges and preparation differ • All users must understand the Saudi Arabian certificate requirements for Hajj and Umrah • All users must previously have used PGD's to supply or administer medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medicinal products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation against invasive meningococcal disease caused by <i>Neisseria meningitidis</i> groups A, C, W-135 and Y, in individuals at increased risk through travel or circumstance. Administration is by a registered healthcare professional working to this PGD.
Inclusion criteria	• Age within the licensed range of the specific product being used. This differs by product and must be checked: Nimenrix from 6 weeks, MenQuadfi from 12 months, Menveo from 2 years. There is no upper age limit for any of the three.

	• Travel to Saudi Arabia for Hajj or Umrah. Saudi Arabia requires proof of MenACWY vaccination as a condition of entry, for Umrah at any time of year as well as for Hajj. • Travel to the sub-Saharan African meningitis belt, particularly for long stays, close contact with the local population, or travel in the dry season from December to June. • Adolescents and young adults up to their 25th birthday who missed the routine adolescent NHS offer, including university entrants. • Asplenia, splenic dysfunction, or complement deficiency, including individuals due to start a complement inhibitor such as eculizumab, where vaccination should be at least 2 weeks before treatment starts. • Occupational risk, for example laboratory staff who may handle <i>Neisseria meningitidis</i>. • Valid informed consent obtained. • No exclusion criteria present.
Exclusion criteria	• Age below the licensed minimum for the product held. Nimenrix is the only product licensed below 12 months, and Menveo cannot be used below 2 years. • Confirmed anaphylactic reaction to a previous dose of the same vaccine, or to any of its excipients or manufacturing residues. • For Menveo specifically: hypersensitivity to diphtheria toxoid or CRM197, which is its conjugate carrier. Nimenrix and MenQuadfi are conjugated to tetanus toxoid instead, so this exclusion is product specific. • Acute severe febrile illness. Postpone until recovered. Minor illness without fever is not a reason to defer. • Formal public health management of a case contact or an outbreak. This is directed by the local UKHSA Health Protection Team and is outside the scope of a private travel PGD. Refer.
Cautions	• Pregnancy. The Green Book position is that meningococcal vaccines may be given in pregnancy when clinically indicated, and there is no evidence of harm from inadvertent vaccination. The product SPCs are more cautious and advise use only when clearly needed. Document the risk assessment and the discussion. • Breastfeeding. Excretion in breast milk is unknown for all three products. Vaccinate where the benefit outweighs the theoretical risk and record the decision. • Immunosuppression, including HIV. Vaccinate as indicated, but the response may be sub-optimal. Individuals with complement deficiency or on a complement inhibitor remain at increased risk of invasive disease even after successful vaccination, and must be told so rather than reassured. • Bleeding disorders, thrombocytopenia or anticoagulation. Use a fine needle, apply firm pressure without rubbing for at least 2 minutes, and warn the patient about the risk of haematoma. • Previous MenC conjugate vaccine only. This gives no protection against groups A, W and Y. Treat the patient as MenACWY naive; a previous MenC dose does not shorten or alter the schedule. • Previous plain polysaccharide meningococcal vaccine. Antibody responses to a subsequent conjugate dose may be lower than in a vaccine-naive person. This does not change the dose but is worth

	recording. Waning group A antibody. Where an individual had MenACWY more than approximately one year ago and faces particular ongoing group A risk, a further dose may be considered.
Anaphylaxis and observation	- Adrenaline 1:1000 injection must be immediately available whenever a vaccine is administered under this PGD, together with a written anaphylaxis protocol consistent with current Resuscitation Council UK guidance. - All staff administering vaccines must be trained in the recognition and immediate management of anaphylaxis and must hold current basic life support training. - The patient should remain on the premises for observation after vaccination in line with the pharmacy SOP. - Procedures must be in place to prevent injury from a vasovagal faint, which is common around vaccination, particularly in adolescents.
Actions if patient is excluded or declines treatment	- Discuss the reason for exclusion with the patient and ensure they understand it. - Advise on alternative options and how these can be accessed, for example the GP, a travel clinic, or a specialist service. - Document the reason for exclusion, the advice given and the decision reached. - Inform or refer to the GP as appropriate, and where the exclusion is time critical make the urgency of that referral explicit to the patient.

Details of the medicine

Name, form and strength of medicine	- Nimenrix (Pfizer), powder and solvent for solution for injection, reconstituted to 0.5 mL. Conjugated to tetanus toxoid. Licensed from 6 weeks of age. - MenQuadfi (Sanofi), 0.5 mL solution for injection in a single dose vial. Ready to use, no reconstitution. Conjugated to tetanus toxoid. Licensed from 12 months of age. - Menveo (GSK), powder and solution for solution for injection, reconstituted to 0.5 mL. Conjugated to CRM197. Licensed from 2 years of age. - Confirm which presentation is held before use. Menveo has been supplied in more than one presentation and the preparation steps differ. Record the product name, batch number and expiry for every dose.
Legal category	POM
Storage	Store at 2°C to 8°C in the original packaging, protected from light. Do not freeze, and do not use any vaccine that has been frozen. Maintain and record cold chain in line with the pharmacy SOP.
Preparation	- Nimenrix and Menveo require reconstitution. Use immediately after reconstitution. In-use stability has been demonstrated for 8 hours but delay is not recommended. - MenQuadfi requires no reconstitution and is drawn directly from the vial. This removes a preparation step and the associated error risk.
Route/method of administration	- Intramuscular injection only. Never intravenous, intradermal or subcutaneous. - Infants under 12 months: anterolateral thigh. From 12 months and in adults: deltoid or anterolateral thigh.

	Where another vaccine is given at the same visit, use a separate limb where possible, or sites at least 2.5 cm apart in the same limb. Record the site used for each vaccine.
Dose and frequency of administration	0.5 mL, whichever product is used. - From 12 months of age and in adults, including all travel indications: a single dose. - Nimenrix only, infants 6 weeks to under 6 months: two doses 2 months apart, with a booster at 12 months if the primary course was completed before 12 months of age. - For Hajj and Umrah the dose must be given at least 10 days before arrival in Saudi Arabia. - Routine booster doses are not currently recommended for most travellers. JCVI has not determined the need for or timing of boosters in at-risk groups such as asplenia or complement deficiency, so do not invent an interval; assess individually and refer where there is doubt.
Quantity to be administered and/or supplied	One dose per patient per attendance, in accordance with the schedule above. This PGD does not permit supply of vaccine to the patient for administration elsewhere.
Adverse effects	Very common: irritability, drowsiness, loss of appetite, headache, fever, fatigue, and injection site pain, swelling and redness. Common: gastrointestinal upset and injection site haematoma. Uncommon: hypersensitivity, dizziness, rash, urticaria, myalgia and malaise. Rare: febrile convulsion. Reported post-marketing: anaphylaxis, lymphadenopathy and extensive limb swelling. Consult the current SPC for the product used for the complete list.
Yellow card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP with whom the individual is registered - name of healthcare practitioner - name and brand of vaccine, batch number and expiry date - date of administration, dose, route and anatomical site used - where a course is involved, the dose number and the date the next dose is due - advice given, including advice given if excluded or if the patient declines - details of any adverse reaction and the action taken - that the vaccine was administered via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed.

Certification for Hajj and Umrah

- Saudi Arabia accepts a quadrivalent conjugate vaccine given within the last 5 years, or a quadrivalent polysaccharide vaccine given within the last 3 years, and in both cases at least 10 days before arrival.
- The certificate must state that the vaccine given was a conjugate vaccine. If the type is not stated, the Saudi authorities treat the certificate as valid for 3 years only, regardless of what was actually given. This single word is the difference between a 5 year and a 3 year certificate.
- The certificate should carry the patient's full name, the vaccine name and type, the date given, and the official stamp of the issuing pharmacy together with the name and registration number of the healthcare professional.

- Advise the patient to keep the certificate with their passport and to carry it for the duration of travel.

Patient information

- Give the patient the manufacturer's patient information leaflet.
- Explain what the vaccine does and does not protect against. It does not cover meningococcal group B, for which a separate vaccine exists.
- Advise that protection is not immediate and, for Hajj or Umrah, that the 10 day rule is a requirement of entry.
- Counsel on the common local and systemic reactions and their self-limiting nature.
- Emphasise that vaccination does not remove the need to seek urgent medical attention for symptoms of meningitis or septicaemia, and describe those symptoms, including that a rash may be a late sign or absent altogether.
- For individuals with complement deficiency or on a complement inhibitor, state plainly that they remain at increased risk despite vaccination.

References

- UKHSA. Immunisation against infectious disease, the Green Book, chapter 22, Meningococcal. Published 10 June 2025, updated 30 July 2025.
- Summary of Product Characteristics for Nimenrix, MenQuadfi and Menveo, current versions, electronic Medicines Compendium.
- Saudi Ministry of Health. Health requirements and recommendations for travellers to Saudi Arabia for Hajj, 1447H (2026).
- NaTHNaC TravelHealthPro. Hajj and Umrah factsheet, and country information pages.
- Resuscitation Council UK. Emergency treatment of anaphylaxis, current guidance.

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
25/8/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

To be completed by the adopting pharmacy: the first block is signed by that pharmacy's superintendent or clinical lead, and the second by its professional group signatory. Get Real Health signs the authorship block below, not these.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		21/08/2026
Chris Pilkington	Pharmacist GPhC: 2046322		21/08/2026

Change history

Version number	Change details	Date
001	New standalone PGD. Separated from the combined Japanese encephalitis, rabies and meningococcal ACWY document following the query from Pharmacy Plus Health on 21 August 2026. Content rebuilt from Green Book chapter 22 and the three current product SPCs rather than carried across, because the combined document did not distinguish between the licensed age ranges of Nimenrix, MenQuadfi and Menveo, did not carry the Menveo CRM197 hypersensitivity exclusion, and did not state the Saudi certificate requirement that the vaccine be recorded as a conjugate.	21st August 2026